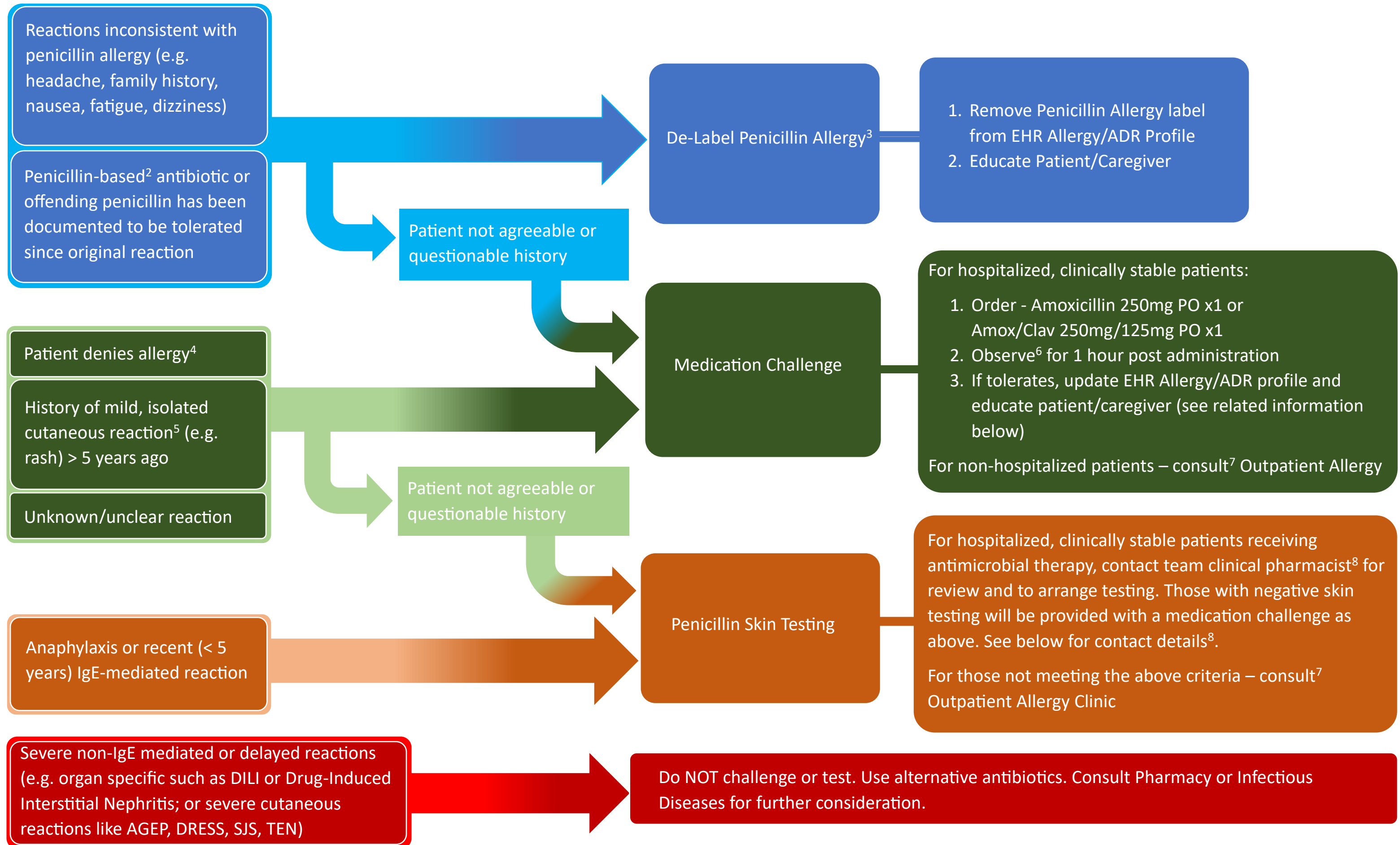


VA Ann Arbor – Penicillin¹ Allergy Evaluation Algorithm

Approved: [Date]



1. Penicillin – a “penicillin” allergy refers only to a documented reaction to “natural penicillins” – i.e. penicillin, dicloxacillin, nafcillin and oxacillin.

2. Penicillin-based antibiotic – refers to the above natural penicillins, but in addition: amoxicillin, amoxicillin/clavulanic acid, ampicillin, ampicillin/sulbactam, penicillin G, penicillin VK, and piperacillin/tazobactam.

3. All clinical staff appropriately privileged may de-label a penicillin allergy in accordance with this algorithm.

4. Clinical judgement required: competent patients who can provide reliable histories and deny an allergy may be considered for directly de-labeling or oral challenge.

5. Isolated reactions – refers to cutaneous reaction history that does NOT include any other accompanying symptom (e.g. systemic symptoms, mucous membrane involvement, respiratory symptoms etc.)

6. Observation and procedure information for medication challenge:

a. This procedure is performed on patients according to the accompanying algorithm. The medication challenge does not require 1:1 nursing.

b. For those who do not require skin testing, the primary team will be responsible for ordering medication challenge (see algorithm for recommendations).

i. Bedside RN will administer medication as directed by order and perform monitoring as follows:

- Obtain baseline vital signs (heart rate, blood pressure, O2 sat, respiratory rate) and at 30-minute intervals for 1 hour post medication challenge.

- If any of the following signs and symptoms are noted during the 60-minute dose challenge monitoring, contact the primary team and the penicillin allergy testing provider and/or initiate rapid response (or call a code) as situation/location dictates.

· New cutaneous reactions, such as hives or rash

· Swelling of the throat, mouth, tongue or difficulty breathing, coughing, wheezing, shortness of breath

· Hypotension – less than 90 mm Hg systolic or symptomatic change from baseline

· Sudden drop in the oxygen saturation below 90% with coinciding shortness of breath or difficulty breathing

· Loss of consciousness.

c. If no symptoms develop after the one-hour observation period, the responsible provider will update the electronic health record medication allergy/adverse reaction profile, inform, and educate the patient or caregiver, complete documentation of the evaluation, and notify the primary team of the outcome (if applicable).

d. If symptoms develop, the penicillin allergy testing provider will update the patient’s beta-lactam allergy/ adverse reaction profile labeling accordingly, and the primary team will be notified of the result.

7. Consult to Outpatient Allergy Clinic via CPRS – Consults/Outpatient/Subspecialty-ALLERGY CLINIC/OTHER: Input reason for request text “Penicillin Allergy Assessment.”

8. Team Clinical Pharmacist contacts: Please contact team pharmacist who can direct the request to an available provider for review and allergy testing for appropriate admitted patients. For general questions, please contact Acute Care Clinical Pharmacy Program Manager (27842) or Allergy Clinic.

Practice Area	Phone	Hours
Ruby Inpatient Medicine Team	5-3725	Mon-Fri
Blue Inpatient Medicine Team	2-2169	0700-1530
Emerald Inpatient Medicine Team, Orthopedic Surgery (Floor), Urology Surgery (Floor), Ob/Gyn Surgery	5-3032	Mon-Fri
	2-2166	0700-1530
Silver Inpatient Medicine Team; Ears, Nose, Throat Surgery (Floor); Gold Inpatient Medicine Team	5-3759	Mon-Fri
	2-2376	0700-1530
Medical Intensive Care Unit, Medical Progressive Care Unit, Neurology, Gold Inpatient Medicine Team, CLC TPNs	5-3762	Mon-Fri
	2-2168	0700-1530

*Floor = non-ICU/PCU i.e. 5E, 5W, 6S, 5N.

Practice Area	Phone	Hours
Surgical Intensive Care Unit, Surgical Progressive Care Unit, Neurosurgery, General Surgery, Vascular Surgery, Cardiothoracic Surgery, CLC TPNs	5-3004	Mon-Fri
		0630-1500
Acute Inpatient Mental Health, Plastic Surgery (Floor), Blue Inpatient Medicine Team	5-3747	Mon-Fri
	2-2240	0700-1530
Emergency Department	2-2852	TBD

*Floor = non-ICU/PCU, i.e. 5E, 5W, 6S, 5N.

Algorithm Abbreviations: ADR – Adverse Drug Reaction; AGEP - DILI – Drug-Induced Liver Injury; DRESS – Drug Rash with Eosinophilia and Systemic Symptoms; EHR – Electronic Health Record (i.e. CPRS); GI – Gastrointestinal; IgE – Immunoglobulin (type) E; PCN – Penicillin; SJS – Stevens-Johnson Syndrome; TEN – Toxic Epidermal Necrolysis